

Stress, Anxiety and Depression

Everything you need to know....

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Learning outcomes

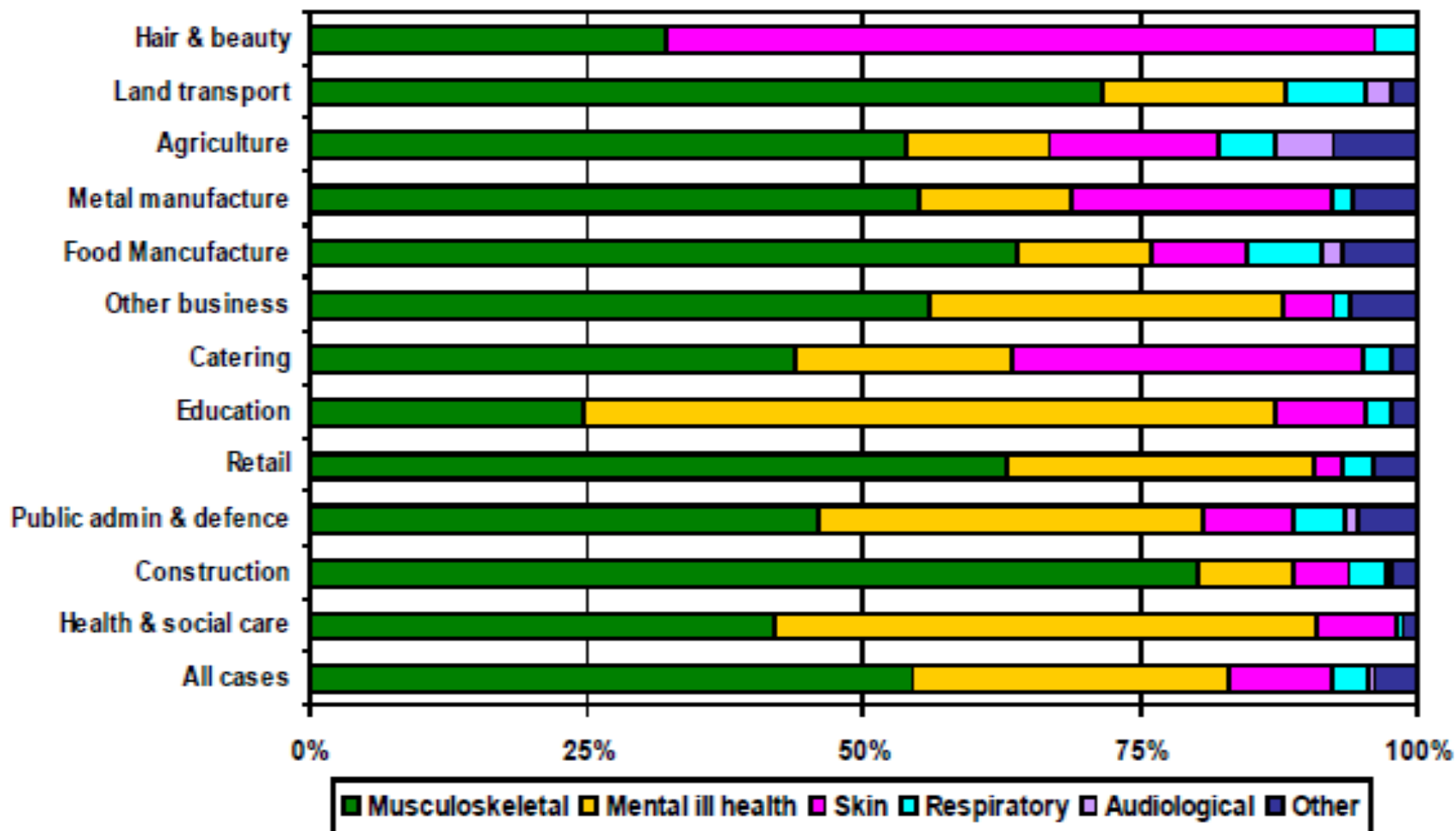
- Introduction to the concept of mental illness
- An approach to evaluating referrals
- To increase awareness of other factors affecting stress, anxiety, mood
- Concept of Occupational Hygiene
- Limitations of medical evidence

Potted biography/disclaimer

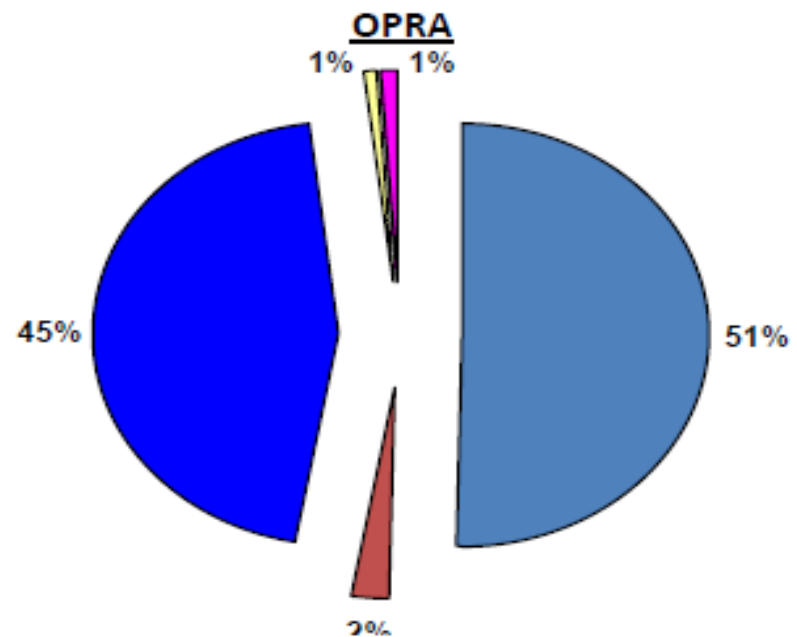
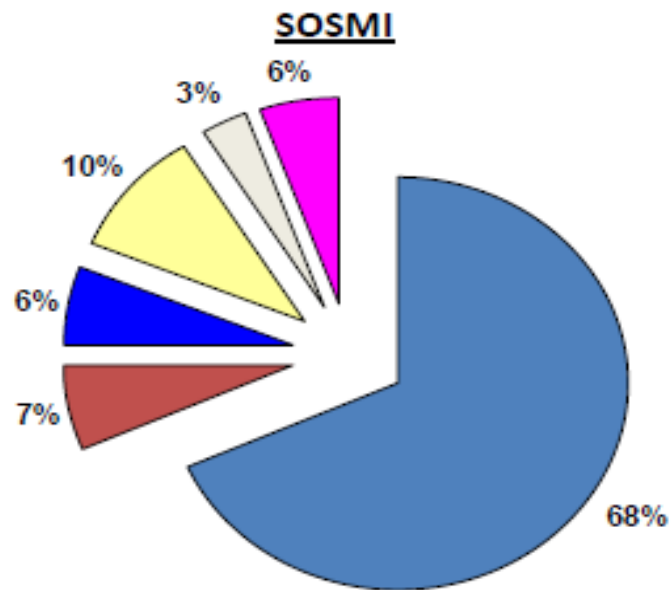
- Qualified 2001
- Member RCPsych 2006
- Close liaison with and teaching of GPs
- Trained in Medical Investigations
- CPD Diploma in Occupational Health 2013
- MSc in Management of Health and Social Services
- Currently NHS Consultant at Royal Oldham Hospital, Pennine Care NHS Foundation Trust since 2010.
- Joined Unum 2013
- Views are my own; not necessarily endorsed by any of the above

What OH Practitioners see

Industries reported by diagnostic category THOR-GP

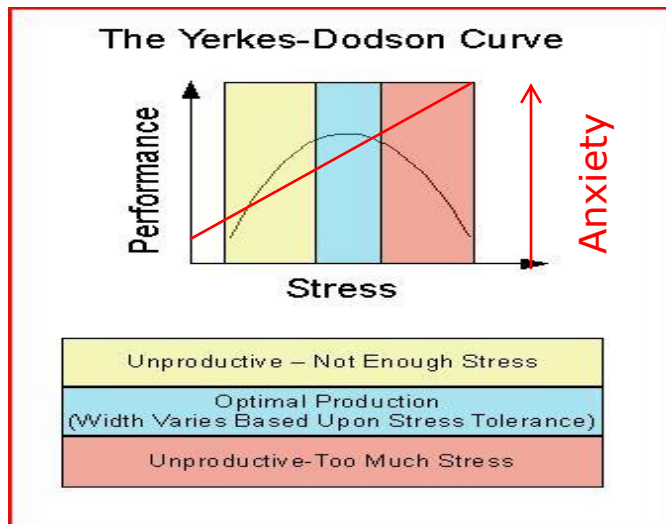


Estimated cases and diagnostic category – SOSMI & OPRA (mental ill health category)

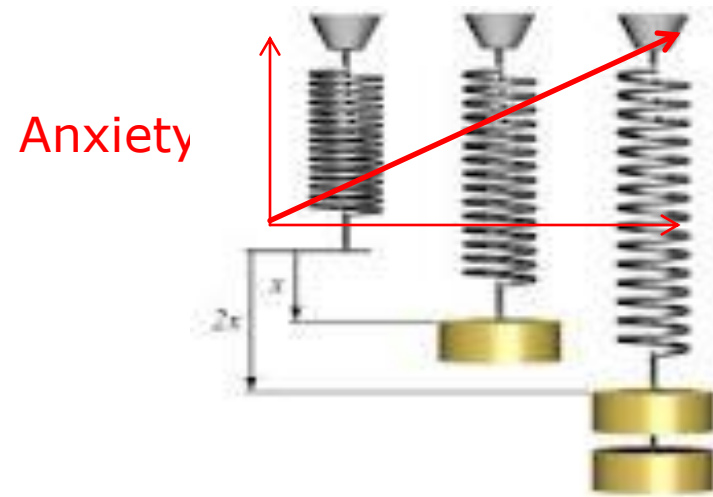


- | | |
|-----------------------------|----------------------------------|
| ■ Anxiety/Depression | ■ Post traumatic stress Disorder |
| ■ Other work Related Stress | ■ Alcohol or drug abuse |
| ■ Psychotic Episode | ■ Other Problems |

Stress – unsatisfactory term



Is associated with arousal or **Anxiety**



Flight / Fight
Bodily sensations
Psychological changes

Stress

UNSATISFACTORY TERM

- Not a diagnosis
- Simply a description
- Highly subjective
- Same demand can lead to stress depending on person's mental health at time
- Expressed as changes in mood, arousal (anxiety), biological rhythms, cognitive function and self esteem.
 -  **Similar to depression!**

Stress

WHY DO WE SEE IT ON MEDICAL RECORDS THEN?

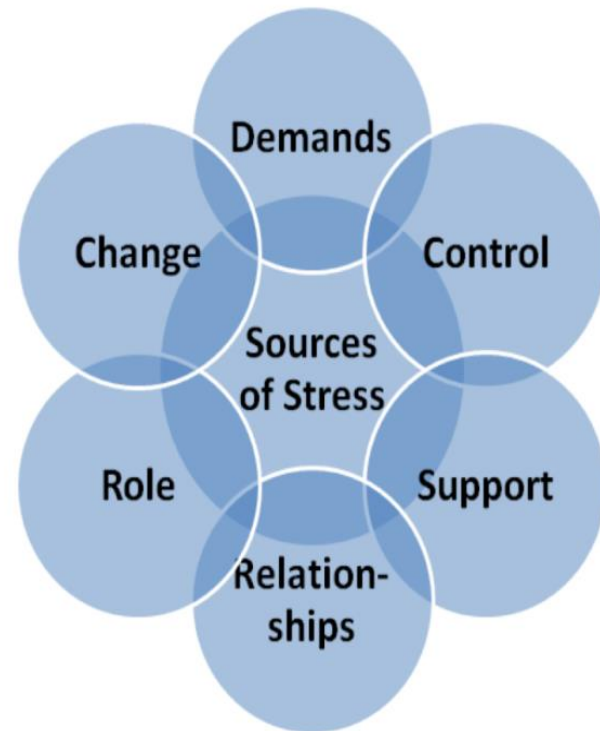
- Picture the scenario in GP surgery (time constraint, note keeping, consultations process, imprecise diagnosis, lack of training in Occupational Health)
- Can be because no mental illness identified
- Can be as a euphemism to spare patient
- If psych calls it work related stress then it doesn't reach threshold for illness
- Remember adjustment disorder
- No strong evidence linking stress to specific disease or illness.

GP's Behaviour - Hussey et al. *BMJ* 2004;328:88-91

- *We are the gatekeepers of [the system], and we don't really understand what happens. "There's a lot of bumf I've had to read over the years, but I never read it".*
- *"Have you ever written something really c**p on a Med 3?"
- "Yeah, I write neurasthenia and I scribble it so even I can't read it, and they have never asked for clarification, so they are obviously happy for you just to scrawl something totally illegible".*
- *"I've no discrimination at all, if a patient comes in and says I need to be off for 2 weeks...with a cold, I'll give him a med 3 no questions asked".*

Stress and the Workplace

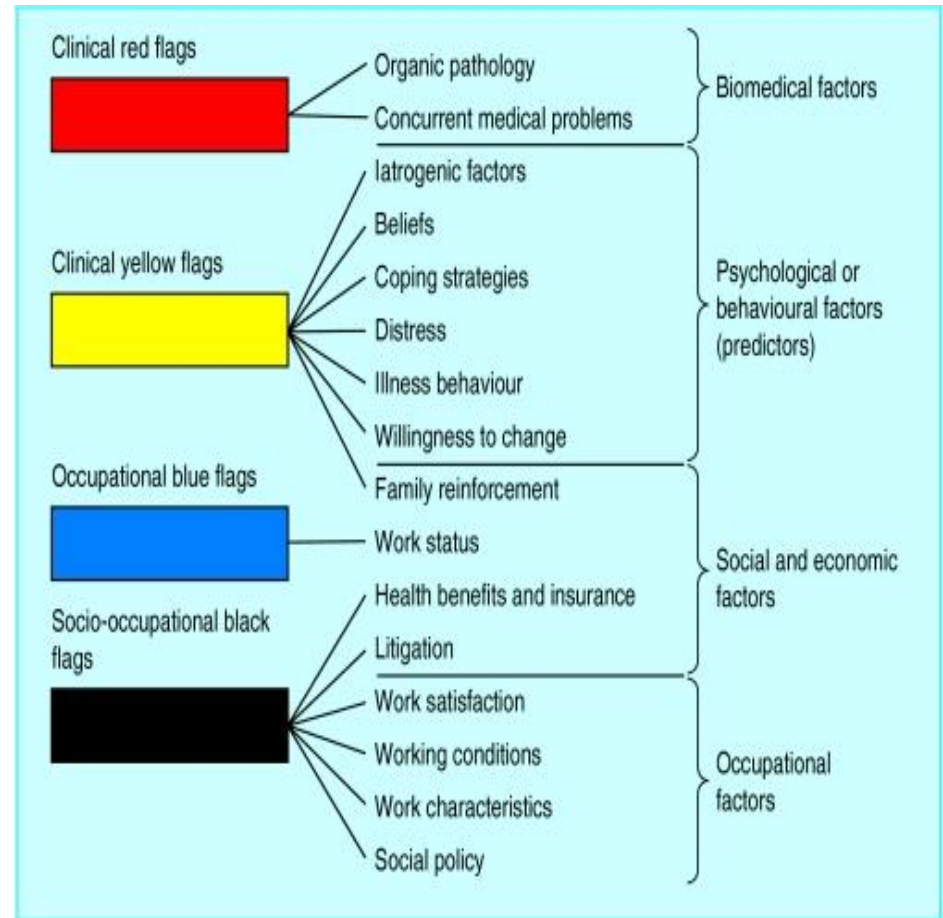
- The Health and Safety Executive have published **Management Standards** defining where the risks from work related stress are being effectively managed and controlled.
- Standards, tools and resources available
<http://www.hse.gov.uk/stress/standards/index.htm>



The Health and Safety Executive
Management Standards Framework

Chronic cases

- Many factors are non-medical and require acknowledgement and non-medical management for successful resolution.
- Normal features of stress can distract from this.



Burnout

- Maslach & Jackson (1982, 1984, 1986) credited with the development of burnout as a "scientific concept".

Three specific types:

- emotional exhaustion
 - depersonalisation
 - decreased personal accomplishment
-
- Some researchers suggest that it may be little more than the experience of stress.
 - There is a diverse theoretical set of models regarding burnout.

Discuss



Evaluating whether it is mental health condition

- Ignore the underlying cause (e.g. work)
- What is the extent of symptoms and effect on functioning *outside work*
- To what extent do symptoms vary from day to day (more severe illness generally unresponsive to circumstances)
- If still affected months after absence from work then probably more than work related stress
- If functioning OK once off work – how quickly did they recover, how much treatment required

Understandable response vs. mental illness

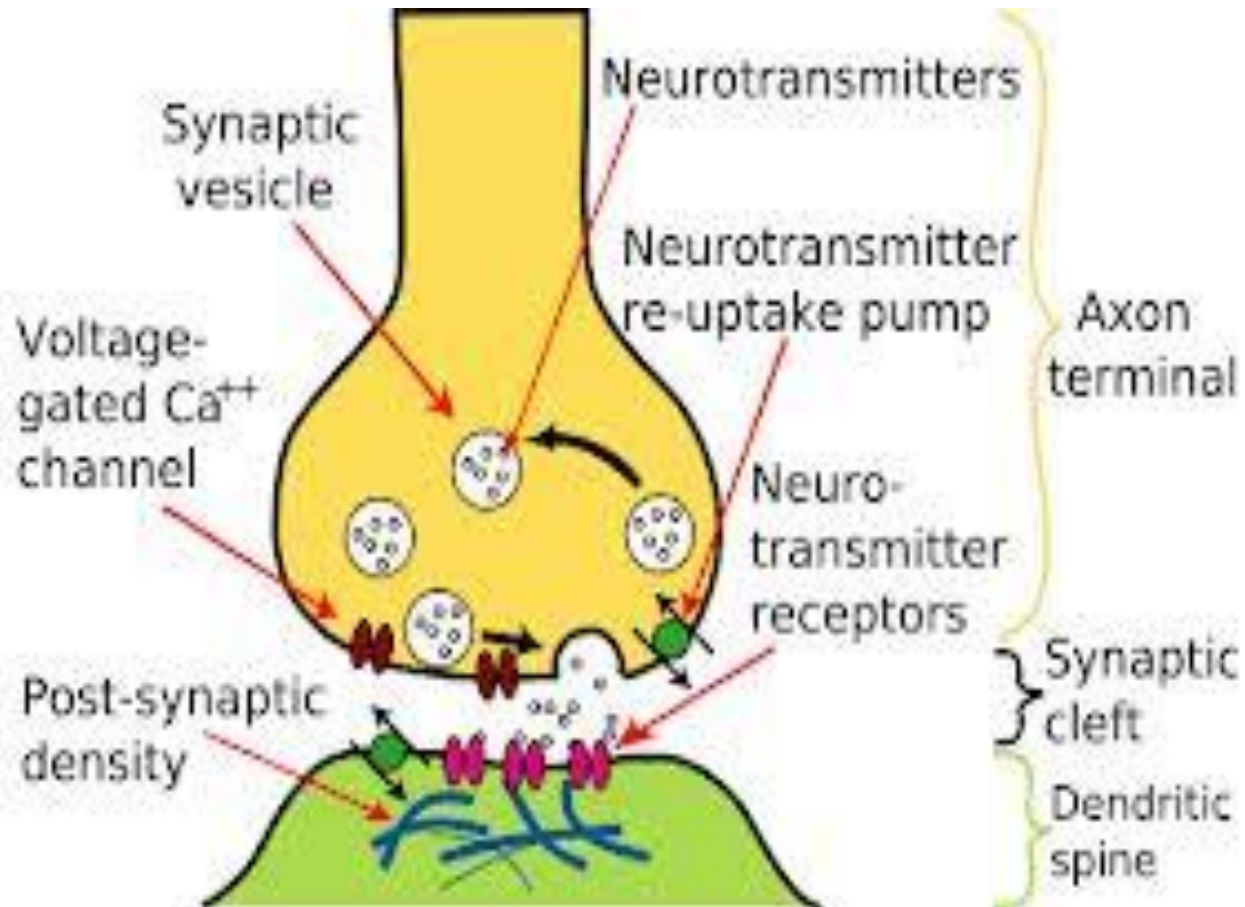
- Look at emphasis in medical records on work issues
- Some GP records of consultation dominated by work complaints
- Others may mention them as initial 'trigger' but then symptoms and wider effect on functioning dominates subsequently
- 'Bullying', line manager difficulties – is this an understandable response or distorted thinking due to mental illness
- Emphasise contrast between functioning at work vs other aspects of life e.g. social, driving, hobbies, holidays
- Look at response of GP e.g. offered medication, suggested work meeting,
- Link in with our Rehab services e.g. stress management workshops
- 'Bullying' is not essential or desirable in any job description...

Symptoms of depression

	ICD-10	DSM-IV
Core symptoms	Depressed mood Lack of interest/ commitment Reduced energy/ increased fatigability	Depressed mood Lack of interest
Additional symptoms	Reduced feelings of worth Feelings of guilt Thoughts about death or suicide Diminished ability to think or concentrate Agitation or retardation Disturbances of sleep Changes in weight and appetite	Reduced feelings of worth Feelings of inappropriate guilt Recurrent thoughts about death or suicide Diminished ability to concentrate or make decisions Agitation or retardation Disturbances of sleep Changes in weight and appetite Fatigue

- DSM-5 contains several new depressive disorders, including disruptive mood dysregulation disorder and premenstrual dysphoric disorder.
- What was referred to as dysthymia in DSM-IV now falls under the category of persistent depressive disorder
- Neither the core criterion symptoms applied to the diagnosis of major depressive episode nor the requisite duration of at least 2 weeks has changed from DSM-IV
- Bipolar II

Clinical depression



Clinical Depression

- Neurogenesis
- Synaptic remodelling
- Animal models

Key discriminators

- Definitely abnormal for the individual (ICD 10 depression),
- largely uninfluenced by circumstance (ICD 10 depression),
- unresponsive to circumstances (ICD 10 depression),
- varying little from day to day (ICD 10 depression),
- not restricted to any particular environmental situation (GAD ICD 10).
- Loss of interests and enjoyment affecting nearly all activities (DSM depression).

Depression

- ICD vs DSM – mild depression does not cause significant functional impairment
- Major depressive disorder=moderate to severe depression
- Most people will have a recurrence in their life time
- Bipolar disorder depression much more significant

Treatments

- SSRI
- Dual Acting agents
- Atypical antipsychotics
- Older agents
- Lithium, T3, ECT, sildenafil, L-tryptophan

- Some are more effective than others
- CBT

Current research

- Ketamine
- Anti-steroid
- NSAID

Evaluation of depression

- With medication – most *begin* to respond after 2-4 weeks
- Full effect after 2-3 months
- CBT roughly equally effective for moderate severity depression.
- Severe depression needs medication/ECT/combo treatment
- Remember generally not responsive to circumstances – if only when discussing work is it dissatisfaction/unhappiness/disagreement

Depression evaluation

- Counselling fine initially...
- Does not *necessarily* need specialist referral but usually the case if no improvement after 6 months

Natural progression

- Mild to moderate: 4 - 16 weeks
- Severe: 30 weeks (25% up to 1 year)
- Recurrent episodes: 4 - 16 weeks.
- Chronic course 10 - 20 % (> 2 years)

Primary versus secondary

- Given "*largely uninfluenced by circumstance* (ICD 10 depression)" etc there are few disorders that are secondary to something else.
- A more nuanced view of the Association between stressor and disorder is necessary.
- Organic presentations
- adjustment disorder
- Phobia
- PTSD

Physical illness and "psychiatric" presentations

- Chronic illness.
- Musculoskeletal problems and chronic pain.
- Chronic fatigue syndrome.
- Cancer.
- Psychoactive substances.
- Medically unexplained symptoms.

Finally... PTSD ☠

- Both DSM and ICD have clear and unambiguous diagnostic criteria.
- Psychological approaches differ from medical approaches.
- Medical criteria, "event or situation (either short or long lasting) of exceptionally threatening or catastrophic nature, which is likely to cause pervasive distress in almost anyone."
- Noreen Tehrani (2004), a well published psychologist specialising in trauma at work, "bullying... victimisation can lead to traumatic stress... gradual build-up of traumatic experiences can lead to a form of traumatic stress"

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